MPH Practicum Day 2026

Gillings School of Global Public Health

When free PrEP medication ran out, visits fell by nearly two-thirds

PrEP use at Hanoi Medical University's Sexual Health Promotion clinic, 2019–2026

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Practicum organization: Center for Training and Research on Substance Abuse – HIV (CREATA-H) and the Sexual Health Promotion (SHP) clinic at Hanoi Medical University in Hanoi, Vietnam

−64%
PrEP service visits
Ratio **0.36** (95% CI **0.18–0.69**)

−69%
New PrEP starts
Ratio **0.31** (95% CI **0.18–0.55**)

83 → 39%
Starts per new tester
HIV-negative testers, before vs. after

Ratio: the model's July 2025 estimate divided by the count expected without the cut. Dispensing fell by a similar 67%.

Monthly PrEP service visits at SHP, May 2019 – June 2026

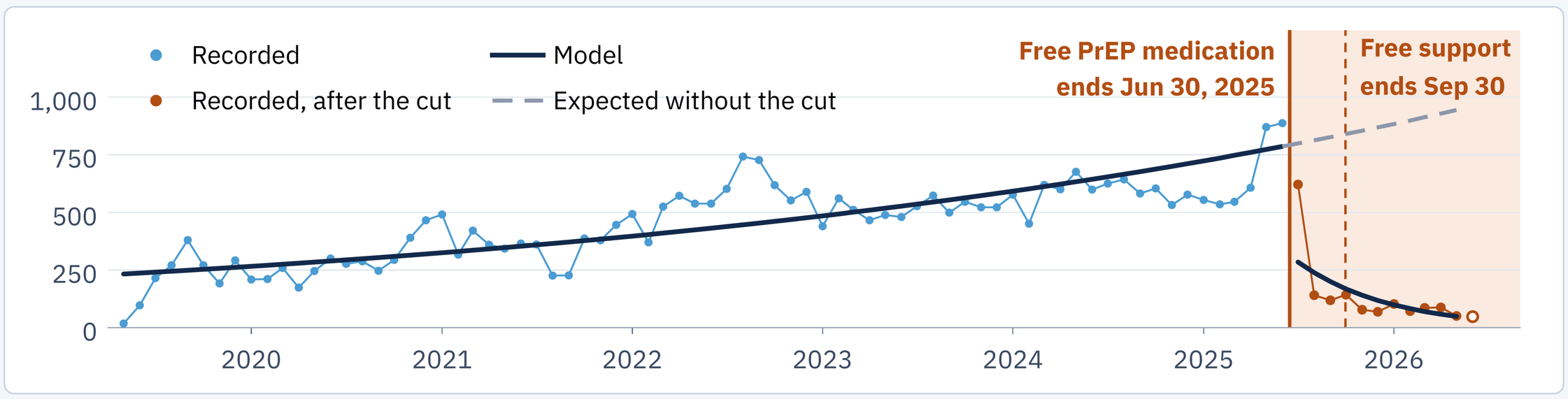


Figure 1. Negative-binomial ITS; a separately funded research cohort excluded. June 2026 partial, not modeled. The July drop is still −64% excluding the May–June stock-up peak, and −55% to −59% if the baseline starts in 2022 or 2024.

01 Background

SHP offered free HIV testing and PrEP with U.S. PEPFAR/CDC support, mostly to men who have sex with men, who accounted for nearly half of Vietnam's new HIV diagnoses in 2020 [1].

After a January 2025 stop-work order, whose waivers left out PrEP [2], free PrEP medication ran out June 30, 2025. Free consultation and support ended September 30, 2025; free HIV testing was set to end in September 2026.

What happened to PrEP use when medication stopped being free?

02 Methods

I rebuilt SHP's reporting definitions from its Stata do-files and applied them to 42,847 visit records from 11,317 clients. The rebuild matched the clinic's own monthly counts in all 10 months checked.

An interrupted time series (negative binomial, Newey–West standard errors) estimated the July 2025 break [3, 4]; clinic staff shaped the interpretation.

03 Results

- A reproducible R pipeline that stops on any failed check
- SHP's first written data dictionary
- A findings memo and briefings for HMU, CREATA-H, and U.S. CDC Hanoi

Starts fell faster than testers

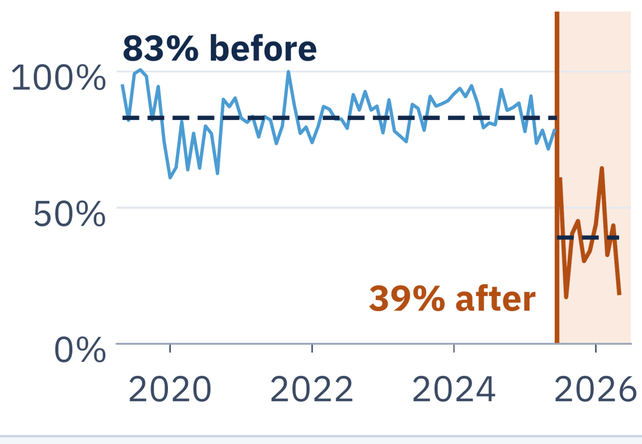


Figure 2. Starts per new HIV-negative tester. Testers fell 53%; starts fell 77%. Monthly flows, not individual conversion.

What this can't show: SHP-delivered PrEP only; prescriptions filled elsewhere are invisible, and one series can't isolate price.

04 Impact

For SHP: findings went first to the clinic team, with an R workflow and data dictionary staff can rerun.

For policy
Testing, counseling, and prescribing mostly held; free medication is what broke. Planned Global Fund support had not reached SHP as of August 2026.

What I learned

- Code can run perfectly on a wrong premise. Diffing my R rebuild against the clinic's Stata workflow caught fields that had stopped being used.
- Two staff at different levels independently described a phased cut, which shaped the model.
- Waiting for the clinic's current do-file, instead of guessing what counted as a new start, turned that result from no clear change into a steep drop.

Next: more Hanoi sites and the interviews this practicum couldn't finish.

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[1] UNAIDS. Equal and equitable access to HIV services across Viet Nam. UN Viet Nam; 2022.
[2] Lankiewicz E, et al. J Int AIDS Soc. 2025;28(2):e26423.
[3] Bernal JL, et al. Int J Epidemiol. 2017;46(1):348–55.
[4] Newey WK, West KD. Econometrica. 1987;55(3):703–8.